

Proposed Insert (Front)

FRONT

1 cm
Barcode

hovid

hovid-Clefan Nasal Spray 0.05% w/v

VICLEFF-M

DESCRIPTION

White to off white suspension filled in an amber glass vial crimped with a nasal spray pump and fitted with an actuator and a cap.

COMPOSITION

Each spray delivers:	
Fluticasone Propionate	50 mcg
<i>Preservative:</i>	
Benzalkonium Chloride Solution	0.02% w/v
Phenylethyl Alcohol	0.25% w/v

PHARMACODYNAMICS

Fluticasone propionate causes little or no hypothalamic-pituitary-adrenal axis suppression following intranasal administration. Following intranasal dosing of fluticasone propionate, (200 mcg/day) no significant change in 24h serum cortisol AUC was found.

PHARMACOKINETICS

- Absorption**
Following intranasal dosing of fluticasone propionate (200 micrograms/day), steady-state maximum plasma concentrations were not quantifiable in most subjects (<0.01 ng/mL). The highest C_{max} observed was 0.017 ng/mL. Direct absorption in the nose is negligible due to the low aqueous solubility with the majority of the dose being eventually swallowed. When administered orally the systemic exposure is less than 1% due to poor absorption and pre-systemic metabolism. The total systemic absorption arising from both nasal and oral absorption of the swallowed dose is therefore negligible.
- Distribution**
Fluticasone propionate has a large volume of distribution at steady-state (approximately 318 L). Plasma protein binding is moderately high (91%).
- Metabolism**
Fluticasone propionate is cleared rapidly from the systemic circulation, principally by hepatic metabolism to an inactive carboxylic acid metabolite, by the cytochrome P450 enzyme CYP3A4. Swallowed fluticasone propionate is also subjected to extensive first pass metabolism.
- Elimination**
The elimination rate of intravenous administered fluticasone propionate is linear over the 250-1000 micrograms dose range and are characterized by a high plasma clearance (CL=1.1 L/min). Peak plasma concentrations are reduced by approximately 98% within 3-4 hours and only low plasma concentrations were associated with the 7.8 hours terminal half-life. The renal clearance of fluticasone propionate is negligible (<0.2%) and less than 5% as the carboxylic acid metabolite. The major route of elimination is the excretion of fluticasone propionate and its metabolites in the bile.

INDICATIONS

hovid-Clefan Nasal Spray is indicated for the prophylaxis and treatment of seasonal allergic rhinitis including hay fever, and perennial rhinitis.

CONTRAINDICATIONS

Hypersensitivity to fluticasone propionate or any of the ingredients.

PRECAUTIONS

- Local infections: infections of the nasal airways should be appropriately treated but do not constitute a specific contra-indication to treatment with hovid-Clefan Nasal Spray.
- The full benefit of hovid-Clefan Nasal Spray may not be achieved until treatment has been administered for several days.
- Care must be taken while transferring patients from systemic steroid treatment to hovid-Clefan Nasal Spray if there is any reason to suppose that their adrenal function is impaired.
- Although hovid-Clefan Nasal Spray will control seasonal allergic rhinitis in most cases, an abnormally heavy challenge of summer allergens may in certain instances necessitate appropriate additional therapy.
- Systemic effects of nasal corticosteroids may occur particularly at high doses prescribed for prolonged periods. These effects are much less likely to occur than with oral corticosteroids preparation and may vary in individual patients and between different corticosteroids preparations. Potential systemic effects may include Cushing's syndrome, Cushingoid features, adrenal suppression, growth retardation in children and adolescents, cataract, glaucoma and more rarely, a range of psychological or behavioural effects including psychomotor hyperactivity, sleep disorders, anxiety, depression or aggression (particularly in children).
- Growth retardation has been reported in children receiving some nasal corticosteroids at licensed doses. It is recommended that the height of children receiving prolonged treatment with nasal corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of nasal corticosteroid, if possible, to the lowest dose at which effective control of symptoms is maintained. In addition, consideration should be given to referring the patient to a paediatric specialist.
- Treatment with higher than recommended doses of nasal corticosteroids may result in clinically significant adrenal suppression. If there is evidence for higher than recommended doses being used then additional systemic corticosteroid cover should be considered during periods of stress or elective surgery.
- Ritonavir can greatly increase the concentration of fluticasone propionate in plasma. Therefore, concomitant use should be avoided, unless the potential benefit to the patient outweighs the risk of systemic corticosteroid side effects. There is also an increased risk of systemic side effects when combining fluticasone propionate with other potent CYP3A inhibitors.
- hovid-Clefan Nasal Spray contains benzalkonium chloride which may cause bronchospasm.